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Submitted via email at ostp-ai-rfi@nitrd.gov

Re: Request for Information; Health Technology Ecosystem [CMS-0042-NC] RIN 0938-AV68

On behalf of the AI Healthcare Coalition (AIHC), thank you for the opportunity to contribute to the Centers for Medicare & Medicaid Services (CMS) and Assistant Secretary for Technology Policy and National Coordinator for Health Information Technology (ASTP/ONC) request for information regarding our country's Health Technology Ecosystem. We appreciate the opportunity to engage with CMS and ASTP/ONC as you consider how to best enable patient, provider, payer access to digital health innovation.

Founded in 2019, the AI Healthcare Coalition convenes healthcare artificial intelligence (AI) stakeholders to advocate for patient access to safe, ethically-developed health AI. Our members are AI innovators who have, or are in the process of developing, AI services (or devices) that require U.S. Food & Drug Administration (FDA) oversight and market authorization. Our founding membership includes the first-ever autonomous AI innovator to obtain FDA authorization, as well as companies who have obtained the first-ever Medicare reimbursement for their respective AI services.

As you consider the impact of current and future policies on digital health innovation, we offer these recommendations for your consideration, and hope to be a continuing resource as you consider the complexities of AI deployment in healthcare settings.

<u>Technology Vendors, Data Providers, and Networks</u>

Ecosystem

- ***TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?***

There are several key steps that CMS can take to drive innovation and private sector investment in AI medical services, including:

1. **To ensure timely coverage of innovative AI technologies, CMS should implement President Trump's Medicare Coverage for Innovative Technologies (MCIT) program.**

During President Trump's first term, the MCIT program was improved to provide a transitional Medicare coverage policy for certain medical technologies that receive "breakthrough technology" status under the FDA. President Biden pulled back this policy, and instead finalized a similar policy, entitled the "Transitional Coverage for Emerging Technologies," or TCET, program. While this program makes some positive steps forward by requiring CMS and the FDA to work contemporaneously and iteratively with innovator applicant companies, the agencies have said that very few—approximately four—applications would be eligible for the TCET process annually. This is far too few. We urge CMS to facilitate timely coverage for AI health technologies by finalizing a MCIT-like policy. In addition to increased patient access, such a policy would create confidence for investment in these important technological advancements, aiding commercialization.

2. **To encourage provider adoption of FDA-authorized, rigorously validated AI medical services, CMS should create permanent Medicare payment pathways for AI technologies.**

While some AI healthcare services are currently reimbursed under the Medicare program, there is no clear pathway for clinicians and innovators to obtain reimbursement for AI services. As more AI services are rigorously validated and FDA-authorized, a clear reimbursement pathway—or multiple pathways—is/are needed to ensure that Medicare beneficiaries and clinicians can access these services. Notably, AI services are diverse and varied, spanning multiple medical specialties and incorporating modalities that vary in their use of algorithmic design and machine learning.

For example, in the case of AI services provided in the outpatient setting, we often see such services assigned to a clinical ambulatory payment classifications (APCs) that do not adequately reimburse for provider costs because there is no specific pathway for AI services. While CMS does have a New Technology APC process to allow reimbursement for new technologies in the outpatient setting, that process was not created to facilitate the payment of AI technologies, and often presents too short or low a payment to allow access. For these reasons, we support a 5-year outpatient payment rate for AI technologies. Permanent payment pathways will create certainty for AI technology developers and providers, which would drive investment and accelerate innovation.

3. **CMS should codify its current policy of separate payment for AI services and underlying imaging services.**

CMS recently finalized a policy in its Outpatient Prospective Payment System (OPPS) rule to allow separate payment for the AI service and the underlying imaging service, but we see ongoing challenges in specialty areas where imaging services and payment are bundled (for example, AI that is viewed as computer-aided detection (CAD)). We urge the administration to provide further

guidance in this area to ensure that its separate payment policy, as finalized by CMS in the CY 2023 OPPS final rule, is effectuated.

4. **CMS should update its outdated rules for imaging services by exclude AI services from the OPPS Cap on imaging services.**

Sec. 5102(b) of the Deficit Reduction Act (DRA) requires that "for imaging services […] furnished on or after January 1, 2007, [CMS] will cap the [technical component] TC of the PFS payment amount for the year (prior to geographic adjustment) by the [annual] OPPS payment amount (prior to geographic adjustment)." Congress enacted Section 5102(b), also known as the OPPS cap, to address rapid growth in spending on imaging services, particularly for advanced imaging modalities—such as computed tomography (CT), magnetic resonance imaging (MRI), and nuclear medicine—as compared to growth in spending among less advanced imaging modalities, such as x-ray or ultrasound.

As is clear in the statutory text, the DRA envisioned services such as x-rays, ultrasound, PET, MRI, and CT as such "imaging services," not autonomous AI or other Software as a Service (SaaS) technologies. Further, when CMS implemented the OPPS cap pursuant to Sec. 5102(b), CMS expressly excluded some services because those services encompassed more than the use of imaging technology alone. CMS should recognize an additional exclusion for services for TC-only services for which there is no associated PC, as CMS has previously recognized that codes without a TC / PC split should not be listed on the OPPS Cap List.

5. **CMS should create a pathway for permanent, separate payment for AI services whose NTAP rate has expired.**

Similar to CMS' New Tech APC program, inpatient AI services may apply for and receive an New Technology Add-on Payment (NTAP) payment for 2-3 years if they meet certain criteria. Once NTAP expires, CMS has reasoned that the cost of the technology should be covered by the DRG payment. The industry does not have data as to whether or not this is happening, and we believe that a lack of ongoing separate payment leads to a lack of provider adoption / patient access. This is particularly true given recent CMS proposals that may reduce the time that NTAP status actually applies to a new technology in practice. The ambiguity about reimbursement post-NTAP disincentivizes investment in these tools which can benefit the Medicare program. If appropriately reimbursed, such tools are likely to be adopted by healthcare providers, improve patient care, and create efficiencies.

Technical Standards and Certification

- ***TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?***

ASTP/ONC should rescind the Biden Administration's recently-adopted certification criteria and standards that preclude adoption of FDA-authorized, rigorously validated AI medical services, specifically:

1. **ASTP/ONC should exclude FDA-authorized AI systems from ASTP/ONC from burdensome "predictive decision support intervention (DSI)" regulations for certified electronic health record (EHR) vendors.**

These rules, which were promulgated under President Biden's term, require onerous reporting by EHR vendors—and in turn by those AI systems that run on such vendors platforms. In a May 2024 fact sheet posted by the agency, HHS/ASTP stated that these "requirements are intended to provide users and the public greater information on whether a Predictive DSI is fair, appropriate, valid, effective, and safe (what we refer to as the FAVES quality framework)." Further, HHS/OSTP opined:

> *"The anticipated outcome of such transparency will increase public trust and confidence in Predictive DSIs, allowing users including healthcare systems, clinicians, and patients, to expand the use of these technologies in safer, more appropriate, and more equitable ways. The information this requirement makes available will support users in navigating the market for predictive and generative AI in healthcare. It will also help address numerous challenges, including risks of bias and harm, that such tools can present. This set of information will become a consistently available, industry-wide baseline upon which others can build, standardize, and enhance."*[1]

These grounds are inconsistent with President Trump's EO 14179, "Removing Barriers to American Leadership in Artificial Intelligence," as they present additional regulatory burden for AI developers. Further, they raise the specter of increased compliance liability for both providers and developers, thwarting adoption. At a minimum, for FDA-authorized AI/ML medical devices, ONC should permit reference to FDA submission and approval documents in lieu of satisfaction of the predictive DSI source attribute reporting.

2. **ASTP should work to exclude FDA-authorized AI systems from HHS/OCR's Sec. 1557 nondiscrimination regulations.**

On May 6, 2024, HHS / OCR and CMS finalized updates to regulations implementing Section 1557 of the Affordable Care Act (ACA) to increase anti-discrimination protections for covered health care entities.[2] Among other provisions, this rule established new obligations for providers who utilize artificial intelligence (AI) and other patient care decision support tools. At the time of issuance, HHS noted that this regulation was in furtherance of President Biden's now-rescinded EO on "Safe, Secure, and Trustworthy Development and Use of Artificial Intelligence."

Specifically, the Sec. 1557 final rule applies nondiscrimination principles to the use of new and broadly defined "patient care decision support tools" in clinical care, and requires those subject to the rule to identify and mitigate discrimination when they use patient decision support tools, including automated and non-automated tools, mechanisms, methods, and technology to provide

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[1] U.S. Department of Health & Human Services, Office of National Coordinator. "ONC Health IT Certification Program Resource Guide: Decision Support Interventions Certification Criterion (45 CFR 170.315(b)(11))" (May 2024) available at https://www.healthit.gov/sites/default/files/page/2024-05/DSI-Criterion-Resource-Guide_508.pdf

[2] "Nondiscrimination in Health Programs and Activities," 89 Fed. Reg. 37522 (May 6, 2024)

patient care. As of now, covered entities are required to comply with the new patient care decision support tool requirements, including an ongoing monitoring and risk mitigation responsibilities.

Although stakeholders expressed concerns regarding regulatory duplication at the time that the rule was proposed, OCR expressly declined to exclude FDA-authorized technologies from the scope of the rule. Importantly, while ONC's requirements for predictive DSIs apply to health information technology developers, Section 1557's requirements apply to covered entity users of patient care decision support tools. These requirements create a redundant regulatory burden for AI innovator companies and healthcare providers, especially for those tools that are already subject to FDA review and authorization.

Conclusion

Your efforts to unleash rigorously validated, FDA-authorized medical services coincides with a time of great excitement for health AI. CMS and ASTP/ONC are in a unique position to usher in a few era of innovation, efficiency, and improved patient health outcomes through the rapid development of health AI technologies.

We look forward to working with your agencies to ensure that patients, providers, and innovators continue to see meaningful improvements in patient access to healthcare AI. If you have any questions about our comments, please contact me at cybil.roehrenbeck@ai-coalition.org.

Kind regards,

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